DR Care Solutions

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Life Plan™

This Life Plan™ is completed for all care needs assessments undertaken by a consultant from DR Care Solutions. It's used as a guideline as to the care options available for a specific client(s). It is a central place to capture your preferences around future ageing related care, types of support and living arrangements.

CLIENTS NAME: **BRUCE ERNEST COX and KERRIE ANN COX**
DOB: **18.01.1949 (76 years of age) and 24.01.1952 (73 years of age)**
CLIENT ID: **DRCS 0542**

Issues covered:	**Background and relevant history;** Penelope Eby (Bruce & Kerrie's daughter) reached out to Danielle from DR Care Solutions on Tuesday 4th March 2025 to discuss her parents ageing care and living needs. Penny resides in Canberra and her parents live in Brisbane, however they were heading to Canberra for a couple of weeks for Penny to provide support to her parents given their recent mental health decline. Penny was referred to Danielle by an existing client of DR Care Solutions, Keith Dalgleish. We agreed over the phone that Danielle would organise to meet with Bruce and Kerrie at Penny's home in Canberra on Thursday 20th March 2025, to assess both parent's care needs as well as discussing care, support and accommodation options available to them moving forward. We spent three hours together which included contacting My Aged Care to ensure both Bruce and Kerrie are in the system (Bruce wasn't). We also confirmed that Penny is listed as their nominated representative and that full and comprehensive ACAT assessments have been ordered given the urgency of the situation. Penny was due to follow up with the allocated assessment teams to determine day and time of the assessments. Since Danielle's visit, Penny has provided feedback about the direction in which Bruce and Kerrie wish to head in after our frank and open discussions about what options are available to them. Bruce and Kerrie reside in a retirement community of 320 homes with two club houses. There is a village manager onsite Monday to Friday however not over the weekends.
• Physical health including mobility, continence, • Cognitive health • Sensory (sight, hearing, oral, feet) • Activities of daily living • Emotional & mental health • Risk assessment of the home and modification recommendations • Medications, allergies & vaccinations • External assistance - including Allied Health • Social background, networks and Relationships	

• Likes & dislikes • Cultural linguistic and Spiritual needs • Estate Plan/ Advice and Life Ready Register • Financial Plan/ Advice • My Aged Care • Care and support considerations	They don't have any emergency call buttons in their home. There are onsite bowling greens which Bruce really enjoys using. They have a nice community of likeminded people around them, however, Bruce feels there isn't a real need to remain in the area if he and Kerrie relocate into a supported living environment. They are currently living in Bethania QLD however have advised Danielle, they are happy to return to Mt Gravatt, QLD (the area they lived for most of their lives) or Logan QLD. This will make it easier for the search knowing their preferred location. **About Bruce:** We have received a copy of Bruce's current GP health summary which outlines past and current health conditions. Bruce has several health issues however his major current concern is increased carer stress and burnout with severe anxiety and depression. Bruce has previously had an enlarged prostate and underwent a TURP procedure (Transurethral resection of the prostate). He's had rectal polyps, varicocele (swollen veins in scrotum), sinusitis, and diagnosed hypertension, asthma, oesophageal reflux, diverticulitis, gastritis, degenerative disc disease in his cervical spine, impaired glucose intolerance and COPD (Chronic obstructive pulmonary disease) *inter alia*. Bruce is independent in his mobility and all activities of daily living, including showering, dressing, undressing, toileting and grooming including shaving. He has no incontinence of bowel nor bladder. He has no skin tears, bruising or wound breakdowns. Bruce believes his short-term memory isn't as good as it used to be. His long term is better. His cognitive decline is likely age and stress related rather than any form of dementia. Bruce hasn't been assessed by a Geriatrician. We suggest that both he and Kerrie be seen by a Geriatrician to oversee their overall health and wellbeing as they age. Bruce is woken by Kerrie each morning (quite early). He wakes and has his own routine. They watch television in separate rooms, he reads the daily news on his iPad. Much of Bruce's time is spent as Kerrie's primary care giver. He is suffering severe carer stress with increased mental health issues. He's seeing a separate psychiatrist to Kerrie. Bruce wears glasses all the time (for reading and long distance). He doesn't need nor wears hearing aids. He has a top set of dentures. He regularly sees a podiatrist for maintenance of his nail care, foot health yet they should be looking at his balance too. He frequently visits his medical and other specialists for check-ups.

	Bruce eats well and is a 'meat and three vegetables' type eater. He loves cooking on his Weber BBQ. He doesn't drink alcohol. He did smoke many years ago. He enjoys fish and chips and all dairy foods including ice cream (given he was a milkman). Bruce plays indoor and outdoor bowls on a Sunday and a Wednesday. He really enjoys this activity as it's something completely different from his primary caring role. Bruce loves any sport involving balls. He enjoys all codes of Football and in particular the AFL and Rugby League (The Broncos). He has Foxtel pay television and watches Foxtel Sports Channel. **About Kerrie:** We have received a copy of Kerrie's current GP health Summary which outlines her past and current health conditions. Many years ago, she developed generalised anxiety disorder (GAD) and obsessive-compulsive disorder (OCD). She has also been diagnosed with Osteoporosis, sleep disorder, hypercholesterolaemia (high cholesterol), anxiety, depression, major depressive disorder with obsessive personality trait and schizophrenia. Penny provided some context about Kerrie's background mental health. After having her son Mark 42 years ago, Kerrie developed puerperal psychosis (or postpartum psychosis). It was managed well under the guidance of the same psychiatrist for 37 years. The last five years has seen a decline in her mental health as her psychiatrist retired. Kerrie had her a great deal of faith in this psychiatrist and the medication he prescribed. One of the long-term medications Kerrie was on, was removed and this was when there were noticeable changes to Kerrie's mental health. She has a new psychiatrist who is working slowly to sort her medications. Kerrie's mental health has been declining for the last five years. Her overall health has been trending downwards. Kerrie undergoes ECT (Electroconvulsive Therapy) at New Farm on a regular basis. Bruce has been transporting and accompanying her to and from this. Ideally, wherever Kerrie ends up, it would be good to have a carer (private carer or employed by the home) to transport and accompany Kerrie to these appointments to remove the stress from Bruce. Kerrie has had frequent admissions to Hospital (voluntary and one involuntary) to manage her major depressive disorder which is severe and recurrent. Her admissions have been six weekly more recently. Kerrie hasn't been assessed by a Geriatrician. We suggest that both she and Bruce be seen by a Geriatrician to oversee their overall health and wellbeing as they age.

	Kerrie has been cleared from a dementia diagnosis. She has periods of lucidity and able to articulate her wants and wishes. Her short-term memory is not as good as it used to be. She was able to provide some long-term history of her childhood. Kerrie presented as depressed; however, she sat for almost three hours without getting up. She wasn't fidgeting and she participated well in the discussions. She was interested in understanding what options are available for her (and Bruce). Kerrie's voice was husky and faint. Penny advised that it wasn't like that the previous day however, it comes and goes. We asked Kerrie how she was feeling on the day of the assessment, and she advised she was feeling a bit 'low'. Kerrie mentioned that she has problems swallowing and prefers a softer diet. This should be investigated by a Speech Pathologist. Kerrie's mobility may need to be assessed as well as she's had a few minor falls (slipping from the bed and missing steps). She doesn't use any mobility aids ie: a walking stick or a four -wheeled wheelie walker. We suggest that an OT (Occupational Therapist) assesses her mobility and determines if this is needed. It's likely to give her confidence while ambulating. She walks slowly and a bit 'wobbly' on her feet. Falls need to be avoided. Penny mentioned that Kerrie (on occasions) does the 'Thorazine Shuffle' (This refers to a movement disorder and distinctive gait often seen in individuals taking certain medications or dealing with neurological conditions like schizophrenia). Kerrie hasn't driven for the last two years. Kerrie bruises easily and has occasional random sores on her limbs. She has no skin tears or open wounds. Kerrie is independent in activities of daily living. She is fully continent of bowels and bladder. She takes 'Movicol' and Coloxyl to keep her bowels regular. Kerrie wakes very early (4.30am-5am) and heads in to wake Bruce. She continues to pace backwards and forwards to his room until he gets up. Given Kerrie is awake so early, she heads to bed around 6.15pm. Overnight, every couple of hours, she's awake, heads to the fridge to find a snack and then heads back to bed. This is the process throughout the night. Kerrie lacks occupation and activities of interest (physical and cognitive) during the day; she becomes bored by watching television and is then awake on and off all night. This would alter if she were in a supported living environment. It will take the pressure off Bruce as well.

	Kerrie has previously had BCC (Basal Cell Carcinoma) and is required to see a dermatologist on a regular basis. The BCC was on her left shoulder previously. Kerrie used to weigh around 42kg. She was always very finely built. Over the last couple of years, she's been snacking more often and has put on about 10kg. She used to love walking, playing tennis and later in life was a 'door to door' saleswoman selling products which included a lot of walking. Kerrie wears glasses all the time. She wears and manages her bilateral hearing aids without difficulty and has had them for the last 10 years. She has a top set of dentures. She regularly sees a podiatrist for maintenance of her nail care, foot health yet they should be looking at her balance too. She visits her medical and other specialists for check-ups on a regular basis. Kerrie snacks all day and night. She drinks tea with milk and one sugar. She doesn't drink coffee. She no longer drinks diet coke however she used to on a regular basis. She's eating cereal regularly (Cheerios, weetbix and Crunchy Nut Cornflakes). She loves ice-cream, biscuits and chocolate. She has morning and afternoon tea. She enjoys soft meat, chicken and fish however only eats a small number of vegetables and salad. She enjoys fruit and berries. Kerrie used to be a good cook however she said she never enjoyed cooking. Kerrie doesn't drink alcohol and has never smoked. Kerrie enjoys playing cards (Canasta, 500 and Bolivia) with a group of four from the retirement community on a Wednesday afternoon. She participates in an activity group once a week with 8 people as part of her social support in her Commonwealth Home Support package. Kerrie used to play in door bowls twice a week however, she's lost interest, and the skills required for this. When Kerrie was asked what brings her joy, she said "knitting, playing cards and watching true crime shows". She also mentioned her children and grandchildren however, she said she finds it hard communicating with her grandchildren. Kerrie enjoys following the Royal Family and their antics and activities. Bruce and Kerrie sleep in separate rooms. Kerrie may require some melatonin (natural sleeping supplement), activities and occupation throughout the day to help her get a better and longer sleep. **Risk assessment of the home and modification recommendations:** Given we were at Penny's home in Canberra, we were unable to assess or determine any risks at Bruce and Kerrie's retirement home in Brisbane.

	As the home has been purpose built, Penny advised there are rails in the shower. The doorways are standard sized, and the house is open plan apart from the bedrooms. There is no emergency alarm system set up. Other things to consider include: all floor rugs should be rolled up and removed to prevent any trip hazards. We also suggest using Plug In Sensor lights LED Plug-In in the hallways and bathrooms as an alternative to turning on lights at night. **Medications, Allergies and vaccinations:** **Bruce** is fully immunised including the Flu Vaccine for 2024. Bruce is allergic to **codeine.** All Bruce's current medications are in their original boxes. He administers them himself. We suggest they are placed into a Webster-Pak® prior to his entry into a supported living environment. Bruce's current medications include: • Amlo (to treat angina and lower blood pressure) • Aristocort (topical cream applied to reduce itchiness, swelling and redness on the skin) • Circadin (to regulate sleep) • Fluvoxamine (an anti-depressant to treat depression) • Irbesartan (to treat high blood pressure and heart disease) • Januvia (to treat Type 2 diabetes & manage blood sugar levels) • Levothyroxine (to treat hypothyroidism) • Metoprolol Tartrate (to treat high blood pressure and angina) • Mirtazapine (to treat depression) • Molnupiravir (to treat mild to moderate Covid 19 - 5 days only) • Neo-B12 Injection every 12 months (B12 supplement) • Ondansetron (to prevent nausea and vomiting) • Panadol Osteo (to treat persistent pain from osteoarthritis) • Serepax (to treat anxiety disorders) • Seroquel (Atypical anti-psychotic to treat bi-polar disorder) • Simvastatin (to lower cholesterol) • Somac (to treat reflux, helicobacter pylori and ulcers) • Symbicort Rapihaler (puffer for Asthma) • Ventolin (puffer for treatment and maintenance of Asthma) **Kerrie** is fully immunised including the Flu Vaccine for 2024. She has no known allergies. Most of Kerrie's medications are in a Webster-Pak®

Many older Australian's have issues pushing out the medications from the Webster-Pak® therefore we suggest purchasing this little cup to support easy removal. Pil-Bob® Medication Removal | Webstercare

Kerrie's current medications (which will need to be confirmed before relocating into care as they are regularly adjusted) includes:

- Avanza (to treat depression)
- Coloxyl (to reduce constipation)
- Crestor (to treat high cholesterol)
- Ferro-Gradumet (supplement for iron deficiency)
- Lithium (to treat bi-polar disorder, mood stabiliser)
- Movicol (to reduce constipation)
- Nortriptyline (a tricyclic anti-depressant)
- Olanzapine (atypical anti-psychotic used to treat schizophrenia and bipolar disorder)
- Ostelin Vitamin D (supplement for Vitamin D deficiency)
- Oxazepam (to treat anxiety)
- Prolia Injections (for the treatment of Osteoporosis)

Social background, networks and relationships; Bruce was born in Brisbane and attended primary and high school. He finished in year 10 at around the age of 15 years of age. He undertook an apprenticeship as a mechanic before taking over his father's local milk run after his father passed away.

Bruce is one of five children. He has an older sister, then Bruce and a younger sister and brother (who are twins). All his siblings live in Brisbane. Bruce and Kerrie see his twin siblings every six weeks or so to play cards with them.

Bruce used to enjoy playing cricket, tennis and squash. He enjoyed gardening. He isn't musical however loves listening to 60's music on YouTube on his iPad or iPhone. Often, he puts on his headphones (ear buds) to listen. He loves to sing however he mentioned he's tone deaf.

Kerrie was born in Brisbane and attended primary and high school and she also left in year 10 at around the age of 15 years of age. She is the youngest of five children. Two of her sisters have passed away and one brother, Bevan passed away about 3 years ago. Her brother, Greg (80) is still alive and lives in Brisbane. They catch up at Christmas and Easter generally.

Kerrie advised that she had a happy childhood and got on well with her siblings. Kerrie didn't play sport at school, and she wasn't musical.

	After leaving school, Kerrie worked for the Commonwealth Bank as the Bank Managers Secretary until she was married. Kerrie also loves 60's music. Kerrie was an avid reader of Mills and Boon books, Danielle Steele and the like. She has difficulty reading due to reduced concentration span. She has a five-minute memory for what she's read. She enjoys watching real crime shows and used to enjoy gardening. Kerrie and Bruce met at a dance hall in 1968. They were married on 30th September 1972. Kerrie and Bruce have three children. Brett (born in 1978) was adopted and has been estranged from the family since 2007, Penny (born in 1982) was also adopted by Bruce and Kerrie and then 13 months later, Mark (1983) was born to Kerrie. Penny lives in Canberra currently, however, has just arrived in from Illinois, USA. She is married to Marc Eby, and they have three children: Thomas (7), Abby (5) and Samson (3). It's a very busy life for them. They have a three-year posting and will be returning to the US in July 2027. Mark is a US Citizen, and their three children are American. Penny comes from an army / defence background and Mark works for the Department of Defence. They have a beautiful defence home in Coombs ACT. There are lots of stairs and split levels which is not ideal for Bruce and Kerrie although there is separate accommodation on the ground floor of the house Mark lives in Meadowbrook, Queensland which is about 15 minutes from Bruce and Kerrie. **Cultural, linguistic and spiritual needs; Bruce** is not religious, and he can't speak any other languages. He mentioned he is spiritual. **Kerrie** is Lutheran and her children went to the Lutheran School in Brisbane. Kerrie used to attend church regularly although finds it too hard to speak with the congregation / community there. Kerrie can't speak any other languages. **Personal alarms and monitoring;** If Bruce and Kerrie relocate into an assisted / supported living environment (Residential Aged Care), they won't require any personal alarms or monitoring equipment (ie: a watch with falls detection and GPS tracking). However, if Bruce were to remain living alone in his retirement village home, we would suggest that he purchase an **Apple Watch** (series 7 and above) or something similar. If something happens to Bruce unexpectedly, Penny would be advised as would emergency services. The home is likely to have an emergency bell however, if Bruce is out of the home, he would need a medical alert and monitoring.

	The watch would need to have **GPS tracking and falls detection** with five emergency contacts listed (including the emergency services). They have a spare key in their meter box (unlocked) for emergency services. The watch should be worn in the shower and while sleeping and charged first thing in the morning when Bruce is fresh and feeling well. **Estate planning;** We checked in with Bruce, Kerrie and Penny to determine if they have Powers of Attorney (for financial decisions should either of them lose capacity) and Enduring Guardianship (for health, care and accommodation decisions should either lose capacity) documentation in place. We have received an e-copy of Kerrie's Power of Attorney (which is listed as Bruce) dated October / November 2013 and then alternates are Penny and her brother Mark. We understand that Bruce has Kerrie listed as his Power of Attorney and Penny and Mark are alternates. This needs to be reviewed. We've received a letter from Kerrie's Consultant Psychiatrist, Dr Jon Steinberg (04.03.2025) who has advised that Kerrie has lost capacity to make decisions in relation to her financial and health situation, therefore Bruce is now enacted to make decisions for both him and Kerrie. This has put enormous pressure on him at an already very stressful time. We've suggested that Bruce and Penny, follow up with the lawyer to have Bruce's Power of Attorney updated and to check that both Bruce and Kerrie have Enduring Guardianship in place (for health, care and accommodation decisions). The four main documents for an estate plan that we recommend are a will; Powers of Attorney (for financial decisions should either lose capacity), Enduring Guardianship (for any health, care or accommodation decisions should either lose capacity) and an Advanced Care Directive. For more information on Estate Planning for families, check the Estate Planning piece of the puzzle here: Aged Care Puzzle | DR Care Solutions | **Advanced Care Directive:** Below we include a special DR Care Solutions link to Touchstone Life Care for building an Advanced Care Directive. You are welcome to use this or ask your GP for a paper-based document if it's preferred. This is usually stored in the client's GP Health Summary and hospital records should something occur, the medical profession adheres to the requests if the person is unable to communicate their wants, needs and wishes. Touchstone Life Care - A DR Care Solutions Partner

	Life Ready Register: For DR Care Solutions clients, we provide a copy of the Life Ready Register, a depository to keep all your affairs in one place. We suggest that you download this and start completing it. No matter what age, everyone can commence building this out and sharing it with their trusted network, their Powers of Attorney and Enduring Guardians as well as their Executor of their Will. We often find the adult children of our clients using this document. Life Ready Register™ **Funeral arrangements:** We suggest considering organising Funerals well before it's required. We believe that Bruce and Kerrie have pre-purchased burial plots at Mt Gravatt cemetery. They don't have any other funeral plans. We work very closely with Funeral Directors (from the Family Business Sector) and would be more than happy to introduce you to businesses that cover your area if requested. Be mindful of payment plans when it comes to funerals, as often they cost more than a standard funeral. Preplanning eases your family's burden during an emotionally difficult time. If you have specific questions, please feel free to ask. If Bruce and Kerrie relocate into care, the selected home will want information on who their selected Funeral provider is. **Financial planning;** Bruce and Kerrie are on full pensions. They own their own two-bedroom, one office home in a retirement community. They have a small amount of superannuation and a term deposit. They will be RAD (Refundable Accommodation Payment) payers if they both relocate into an assisted living environment. Bruce and Kerrie do not have an accountant nor a financial adviser. We will introduce Bruce and Penny to an Aged Care Accredited Financial Adviser to determine the best way to fund Bruce and Kerrie's aged care moving forward ie: lump sum, partial lump sum, daily fees and have a budget provided. Mark is meeting with an accredited financial adviser in the next week or so to determine a high level budget for Danielle to work within. Residential Aged Care is means tested. Centrelink / Services Australia assess the client's capacity to pay this and advise the selected home. The Aged Care Accredited Financial Adviser can assist with the application to ensure you are only charged the amount you are required to pay. For more information on how the Government Support works, please review this section on our website. Government Support | DR Care Solutions.

	My Aged Care; As mentioned earlier in this plan, on the day of the assessment (20.03.2025) My Aged Care was contacted to ensure both Bruce and Kerrie are in the system (Bruce wasn't). We also confirmed that Penny is listed as their nominated representative and that full ACAT assessments have been ordered due to the urgency of the situation. Penny was to follow up with the allocated assessment teams to determine day and time of the assessments. Bruce's assessment has been allocated to *The Department of Health* (ph: 1300 130 143) and Kerrie's re-assessment has been allocated to *Suncare* (ph: 1300 004 622). Previously Kerrie had an entry level assessment for Commonwealth Home Support Programme and was offered minimal services consisting of an activity day out each week for Kerrie (shopping centre, outing) with 8 other people as well as 'meals on wheels provision'. We have **urgently requested ACAT assessments** for both Bruce and Kerry. **NB: We require the codes for Residential Respite Care and Residential Permanent Care to move forward to implement this plan.** Bruce and Kerrie have decided that together they will be relocated into a supported living environment. To keep a record of the family's AC (Aged Care) numbers, we are listing them out in this plan. **My Aged Care phone number is: 1800 200 422** **Bruce: AC#: 778 362 11** **Kerrie: AC#: 168 562 21** **Penny: AC#: 860 085 05** We spoke with 'Ray' from My Aged Care. The reference number of the call is: **2 – 158 725 611 412** **Care and Support considerations:** We discussed the options available to Bruce and Kerrie which included the option of increasing support at home and having a combination of Government Funded Services in conjunction with privately funded, fee for service services. Bruce was hesitant on this option due to the long wait times and minimal funding likely to come though for them. He is feeling extremely overwhelmed, anxious and exhausted. Another option would be for Kerrie to relocate into an assisted living environment (Residential Aged Care) and for Bruce to remain in their retirement home. Kerrie was quite concerned about Bruce being on his own even with support coming in. She was worried about how they would fund her Aged Care given he is currently living in the home (as a protected person).

	Kerrie advised she would like to relocate into a supported living environment. She is very ready; however, she's been worried about Bruce. Given Bruce's current mental health and carer burnout, we suggested that he relocate along with Kerrie into the same environment. He hadn't thought about this previously. We saw the relief on Bruce's face when we advised him that it was possible to move them together. The selected home would be a supported living environment with **care and support onsite.** After the family discussions, it makes sense for them both to relocate together into care given the long wait times for home care and the minimal amount of Government Assistance available to Bruce. Bruce and Kerrie sleep in separate rooms. Finding two separate rooms in a residential aged care home (supported living environment) will be easier than finding a couple's room. We will need to locate a home that has experience dealing with residents **with mental health issues** as it's not the same as dementia care. Some homes are very experienced in mental health, it's just a matter of locating them. We will ensure we find a home that can manage Kerrie and Bruce's health and care, currently and into the future, right through to end of life care. We don't want them to move again. A home which is near to a bowling club would be an added advantage for Bruce.
Goals (what does the client wish to achieve): ● Options ● Preferences ● Ideal Outcome	We've outlined the options above and won't repeat them here. The family's decision is to move forward on relocating both Bruce and Kerrie (together) into residential aged care for a period of respite (usually a minimum of three to four weeks) with a view to relocating them permanently. It means they only have one move. The environment they relocate to can manage their care now and until end of life.
Actions required: (for the client to achieve these goals)	1. **URGENTLY** request and follow up **ACAT assessments** (from My Aged Care). As part of ACAT assessment (for Bruce) and re-assessment (for Kerrie), please obtain additional codes for **Residential Respite Care and Residential Permanent Care**. 2. Danielle from DR Care Solutions to commence the process of finding appropriate **residential aged care in the Mt Gravatt or Logan areas of Brisbane, QLD.** This process ***can't commence*** until such time as the ACAT assessments have been completed with the referral codes for residential respite and residential permanent care provided.

	3. Review existing **Estate Planning Documents** including wills. Establish back up (alternate) Powers of Attorneys, Enduring Guardians and Executors. 4. Danielle will introduce **an Aged Care Accredited Financial Adviser** to Penny and Bruce. 5. **Development of an Advanced Care Directive with** specific emphasis on healthcare and accommodation decisions. 6. **Both to complete the DR Care Solutions, Life Ready Register** and provide the completed document to their Next of Kin and Executors. Information provided in this life plan™. 7. Consider setting up a **prepaid funeral** in event of one or both passing. Please advise Danielle for an introduction to one of our Strategic Partners (Family Business) if interested.
Implementation:	**Implementation:** As outlined above, we have been requested to find the right **Aged Care Home** (Assisted / supported living environment) for respite with a view to permanent care for Bruce and Kerrie. We will run a comparative platform report which brings up the top 10 Aged Care homes in a certain area. This platform has every aged care home in Australia included including new homes and any closed homes, non-compliant or sanctioned homes. We work closely with the family to find the right home for them and tour up to **four or five** in a particular location with Bruce, Kerrie, Penny and possibly Mark. It's not just locating a bed for a client, it's their next home. We personally ensure that the selected home is appropriate for the client's needs, wants and wishes within their financial capacity to afford the home and that they are **clinically accepted** by the home. It's not just the family determining the home We undertake all the groundwork, pre-vetting and due diligence of the aged care homes and remove the pressure and stress out of finding the right home at an already emotionally charged time. We walk alongside you for as long as Bruce and Kerrie are in care.
Who will be involved: (Guardian, solicitor, financial planner/ advisers, family members)	● Bruce & Kerry Cox, Penelope Eby and Mark Cox. ● Danielle Robertson from DR Care Solutions ● Aged Care Accredited Financial adviser (Danielle will introduce) ● Estate Lawyer (review of all estate planning documents) ● GP, Geriatrician, Psychiatrists and any other medical specialists

Review of Life Plan and the date the Family need to determine next steps:	Wednesday 26th March 2025

Signature (Consultant):  Date: Sunday, 23rd March 2025

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